

**PERFORMANCE WORK STATEMENT (PWS)
UNITED STATES MARSHALS SERVICE (USMS)
SPECIAL OPERATIONS GROUP (SOG)
MEDICAL OVERSIGHT SERVICES**

1.0 General: This is a service contract to provide medical oversight, physician-directed provider certifications and ongoing operational and training Contractor Support to the USMS SOG Medic Program. The Government shall not exercise any supervision or control over the contract service provider performing the services herein. The contract service provider shall be accountable to the Contractor who, in turn, is responsible to the Government.

1.1 Scope: The Contractor shall provide all personnel, equipment, tools, materials, supervision and quality control measures necessary, except those specified in Paragraph 3.0 as Government Furnished, to perform all duties specified in this PWS.

The Contractor shall be a university-affiliated academic medical center with faculty physicians and medical staff proficient in diverse operational disciplines such as tactical emergency medicine, K-9 medicine, Emergency Medical Services (EMS), disaster medicine, and mass gathering and event medicine. The university-affiliated model allows the Government to benefit from the academic, clinical and research culture inherent to these institutions. Aligning the SOG Medic Program with academic physicians has allowed the SOG Medic Program to learn and adopt innovations, techniques and pharmacology through the expertise and instruction of career medical educators. The successful Contractor:

- Must have a demonstrated longitudinal track record of practical experience and expertise in tactical medicine administered under a formal and established Special Operations program.
- Must have a documented recent research publication (previous 24-months) specific to tactical medicine.
- Must be actively engaged in the subspecialty training of emergency physicians in this discipline through advanced fellowship programs in Tactical Medicine and EMS.
- Must be located within 50-miles of SOG's Washington, DC or Tomball, TX location. The mileage location is in effect to reduce the Government's travel cost for required meetings and trainings.
- Must have a minimum of twenty (20) years of formal documented work experience in tactical emergency medicine with a focus on law enforcement support and medical direction (preferably in special operations setting).

Contractor support will allow a full range of support services including, but not limited to, medical program oversight of approximately twenty-five (25) SOG medics, provide annual training in tactical and operational medicine, and provide emergency medicine physician access 24-hours a day, seven (7) days per week.

1.2 Objectives: To provide medical oversight, physician-directed provider certifications and on-going operational and training support to the USMS SOG Medic Program.

1.3 Background: SOG is a specially trained element of the USMS responsible for conducting complex and sensitive operations intended to further the rule of law. As part of these operations, SOG personnel are deployed in high-risk and sensitive law enforcement situations, national emergencies, civil disorders, natural disasters and other operational situations as required. The SOG Medic Program provides high-threat emergency medical care. This care is provided during operational deployments, but can also take place at any SOG location. SOG medics attend to both Agency employees and the public as required.

1.4 Period of Performance (POP): The POP shall consist of one (1) base period of 12-months, four (4) 12-month optional periods executable at the sole discretion of the Government, and one (1) not-to-exceed 6-month extension in accordance with FAR 52.217-8.

DATES	
Base Year	01 July 2026 - 30 June 2027
Option Year 1	01 July 2027 - 30 June 2028
Option Year 2	01 July 2028 - 30 June 2029
Option Year 3	01 July 2029 - 30 June 2030
Option Year 4	01 July 2030 - 30 June 2031
FAR 52.217-8	01 July 2031 - 31 December 2031

2.0 General Information:

2.1 Place of Performance: Contractor shall provide the majority of services at either the Contractor's location or any of the various SOG locations. On occasion, circumstances may dictate that training takes place at an alternate location. The Contracting Officer Representative (COR) will notify the Contractor in writing if this applies.

2.2 Inclement Weather, Government Closures and Unscheduled Gate Closures: There are certain types of irregularly occurring situations that may prompt the Government to close offices without advance notice where Contract Personnel are working on either a national, state, local or site-specific basis. A closure may occur at any time before or during the workday. The duration of such closures may range from an early closure with normal operations expected on the next business day to a period of indeterminate length. Such closures may be declared by the President, Office of Personnel Management (OPM), Office of Management and Budget (OMB), the USMS Director, or other appropriate executive authority. Examples of such situations include, but are not limited to: Government shutdowns, Presidential or agency-declared states of imminent danger (e.g., bomb threats); Presidential or agency-declared administrative leave (e.g., Christmas Eve, Inaugural Day); inclement weather (e.g., winter storms, hurricanes, tropical storms, flooding); and infrastructure emergencies (e.g., frozen or burst water pipes, power outages).

Unscheduled gate closures by the Security Police may occur at any time causing all personnel entering or exiting a closed installation to experience a delay. This cannot be predicted or prevented. Contractors are not compensated for unexpected closures or delays. Vehicles operated by contractor personnel are subject to search pursuant to applicable regulations. Any moving violation of any applicable motor vehicle regulation may result in the termination of the contractor employee's installation driving privileges. It is the Contractor and contractor employee responsibility to learn and obey the regulations of the installation; including fire, traffic, safety and security regulations while on the installation.

Contractor employees shall only enter restricted areas when required in performance of the contract and with prior approval of the COR. All contractor personnel are required to carry proper identification at all times and shall be subject to security checks as deemed necessary. The Government reserves the right to direct the removal of a contractor employee from Government property, or to revoke access to Government systems for misconduct, security concerns and/or violations, and any overt evidence of sickness or communicable disease. Removal of contractor employees does not relieve the Contractor from responsibility of total performance of this contract.

2.3 Recognized Holidays*: The following are recognized United States (US) holidays. The contractor shall not perform services on these days:

- 2.3.1 New Year's Day: January 1st
- 2.3.2 Martin Luther King, Jr.'s Birthday
- 2.3.3 President's Day
- 2.3.4 Memorial Day
- 2.3.5 Juneteenth National Independence Day: June 19th
- 2.3.6 Independence Day: July 4th
- 2.3.7 Labor Day
- 2.3.8 Columbus Day
- 2.3.9 Veteran's Day: November 11th
- 2.3.10 Thanksgiving Day
- 2.3.11 Christmas Day: December 25th

*If a holiday falls on a Saturday, the preceding Friday is typically observed as the Federal holiday. If a holiday falls on a Sunday, the following Monday is typically observed as the Federal Holiday. The Contractor is required to verify with the COR what day shall be observed as the Federal holiday if the actual holiday falls on a Saturday or Sunday.

2.4 Quality Control Plan (QCP): The contractor shall develop and maintain an effective QC Plan (QCP) to ensure services are performed in accordance with this PWS. The contractor shall develop and implement procedures to identify, prevent, and ensure non-recurrence of defective services. The contractor's QCP is the means by which it assures itself that its work complies with the requirements of the contract. As a minimum, the contractor shall develop QC procedures that address the areas identified in Technical Exhibit 1, Performance Requirements Summary (PRS). A final QCP shall be submitted to the COR NLT 10 days after contract award. After acceptance of the QCP, the

contractor shall obtain the Contracting Officer's (CO's) acceptance in writing of any proposed changes to its QCP.

2.5 Installation Access and Security Requirements: The contractor shall comply with all applicable installation/facility access and local security policies and procedures, which may be obtained from the COR. The contractor and all associated subcontractor employees shall provide all information required for background checks to meet installation access requirements.

2.6 Special Qualifications: The following special qualifications are required:

- The Contractor shall be an academic medical center with a Level 1 adult and pediatric trauma center with:
 - Stroke center
 - Cardiac intervention center
 - Eye trauma center
 - EMS base station located within the Baltimore-Washington, DC metropolitan area, or within a 50-mile radius of Tomball, TX.
- Contractor shall be capable of allowing SOG Medics the opportunity to observe and participate in "hands on" patient care activities including skills training in the emergency department, urgent care and critical care settings. All training shall be:
 - Under the supervision of physicians, nurses and clinical care technicians as appropriate
 - All clinical training shall take place under the oversight of the designated SOG Medical Director and/or the approved designee(s) in close coordination with the SOG Medic Program Manager.
- The academic medical center must have classroom facilities for a minimum of twenty-five (25) students with both audio and video equipment installed. Classroom(s) must, at a minimum, have the following:
 - Computers for training
 - Internet access for provided computers
 - MS Power Point capability
 - Television
 - Simulation room
 - Conference space
- The Contractor must possess accredited, advanced post-graduate physician fellowship training programs including (but not limited to):
 - Tactical Medicine
 - Emergency Medical Services (EMS)
 - Disaster Medicine
 - Global Health
 - Ultrasound
 - Research and Administration

2.7 Post Award Conference/Periodic Progress Meetings: The Contractor agrees to attend any post award conference convened by the CO or COR. The CO or COR may periodically meet with the Contractor to review the Contractor's performance. During these meetings, the CO and/or COR will apprise the Contractor of how the Government views the Contractor's performance, and the Contractor shall apprise the Government of any problems they are experiencing. The Contractor shall resolve outstanding issues raised by the Government within a mutually agreed to time frame. Contractor attendance at these meetings shall be at no additional cost to the Government.

2.8 Contractor Travel: Contractor travel may be reimbursed in accordance with the Federal Travel Regulations (FTR). For Contractor travel to be reimbursed, the following conditions must be met:

- Travel must be pre-approved by the COR in writing*
- All travel will be subject to the FTR
- Travel must be accomplished utilizing the most economical and discounted form of transportation and lodging available
- Upgrades are not authorized**

* There may be emergency situations where formal travel approval cannot be issued prior to the travel being initiated. In these situations, the COR may issue approval via text message or email.

** There may be specific situations where an upgrade may be allowed to facilitate the requirement. An example would be the COR allowing a rental van in lieu of a compact car to transport materials.

All pre-approved Contractor travel will be reimbursed at cost. Contractor will be required to submit receipts to request reimbursement.

2.9 Need something about PII Security / DATA Rights since contractor will be required to provide consultation on electronic care record and reporting system utilizing a secure USMS server.

2.10 Transition Phase-Out: The Contractor shall follow its' plan for a transition-out period, if the Contractor is not selected for a follow-on award or the Government elects not to exercise any options to extend the performance period. The plan for transition-out shall include, but is not limited to:

- Transfer and/or Return of Government Furnished Property, Material, Equipment, and Data, if applicable;
- Management of Knowledge Transfer Activities via written Standard Operations Procedures, Guidance, or Instructions;
- Ensuring No Disruption of Work and Maintaining Continuity of Operations;
- Monitoring and Reporting of Transition Progress;
- Final Invoice;
- Close-out report that includes the status of funding and all outstanding issues no-later-than 15 days prior to the end of contract performance.

3.0 Definitions and Acronyms:

3.1: Definitions:

3.1.1 Contracting Officer (CO): A person with authority to enter into, administer, and/or terminate contracts, and make related determinations and findings on behalf of the Government. Note: The only individual who can legally bind the Government.

3.1.2 Contracting Officer Representative (COR): An employee of the U.S. Government designated by the CO to monitor contractor performance. Such appointments shall be in writing and shall state the scope of authority and limitations. This individual has authority to provide technical direction to the Contractor if that direction is within the scope of the contract, does not constitute a change, and has no funding implications. This individual does NOT have authority to change the terms and conditions of the contract.

3.1.3 Contractor: A supplier or vendor awarded a contract to provide specific supplies or service to the Government. The term used in this contract refers to the prime.

3.1.4 Contractor Service Provider(s): Individuals employed by the Contractor to provide services under a contract.

3.1.5 Deliverable: Anything that can be physically delivered and includes non-manufactured things such as meeting minutes or reports.

3.1.6 Key Personnel: Contractor personnel that are evaluated in a source selection process and may be required to be used in the performance of a contract by the PWS. When key personnel are used as an evaluation factor in best value procurement, an offer can be rejected if it does not have a firm commitment from the persons that are listed in the proposal.

3.1.7 Quality Assurance: The Government procedures to verify that services being performed by the Contractor are performed according to acceptable standards.

3.1.8 Quality Assurance Surveillance Plan (QASP): An organized written document specifying the surveillance methodology to be used for surveillance of contractor performance.

3.1.9 Quality Control: All necessary measures taken by the Contractor to ensure that the quality of an end product or service shall meet contract requirements.

3.1.10 Subcontractor: One that enters into a contract with a prime contractor. The Government does not have privity of contract with the subcontractor.

3.1.11 Workday. The number of hours per day the Contractor provides services in accordance with the contract.

3.1.12 Work Week. Monday through Friday, unless specified otherwise.

3.2 Acronyms

AQL	Acceptable Quality Level
CDS	Controlled Substance
CFP/M/E	Contractor Furnished Property/Materials/Equipment
CM	Contract Manager
CO	Contracting Officer
COR	Contracting Officer Representative
DEA	Drug Enforcement Agency
EMS	Emergency Medical Services
FAR	Federal Acquisition Regulation
FTR	Federal Travel Regulation
FY	Fiscal Year
GFP/M/E/S	Government Furnished Property/Material/Equipment/Services
IT	Information Technology
MS	Microsoft
MTS	Medical Treatment Facility
NREMT	National Registry of Emergency Medical Technicians
PD	Procurement Division
PII	Personally Identifiable Information
PM	Program Manager
POC	Point of Contact
POP	Period of Performance
PWS	Performance Work Statement
QA	Quality Assurance
QAP	Quality Assurance Program
QASP	Quality Assurance Surveillance Plan
QC	Quality Control
QCP	Quality Control Program
SOG	Special Operations Group
SOLO	Special Operations Low-O
TCCC	Tactical Combat Casualty Care
TE	Technical Exhibit
TECC	Tactical Emergency Casualty Care
TS	Top Secret
USMS	United States Marshals Service

4.0 Government Furnished Property, Material, Equipment and Services: N/A

5.0 Contractor Furnished Property, Materials and Equipment (CFP/M/E): Contractor is required to have malpractice insurance, and all tactical medics are required to have sufficient medical insurance to protect the Contractor against claims of negligence and errors and omissions during care provided in high-risk scenarios.

The USMS recommends that all medical personnel be issued a NIJ certified Level IIIA+ bulletproof vest and ballistic helmet.

6.0 Requirements: The Contractor shall provide all administration, supervision, labor, transportation and other supplies/services in accordance with this PWS. The Contractor shall provide:

6.1 Medical Program Oversight: Contractor will provide oversight for approximately twenty-five (25) SOG Operator Medics. Oversight will include:

6.1.1 Provide physician licensure and authorization for SOG Medics to carry approximately 30 medications from over-the-counter medications to prescription medications including controlled substances (CDS).

6.1.2 Provide a minimum of 40 hours training per year in the latest advancements in tactical and operational medicine, pursuant to the Tactical Combat Casualty Care (TCCC) and/or Tactical Emergency Casualty Care (TECC) guidelines and National Registry of Emergency Medical Technicians (NREMT) certifications. Training must include simulation using realistic injury mannequins, low-light environments, and mass casualty scenarios as well as clinical rotations and field exercises.

6.1.3 Provide off-line medical direction and program administration (minimum of 100-hours per year).

6.2 Provide two (2) Emergency Medicine Physicians on-site for the first five (5) days of all SOG Selection courses. Physicians shall lead, oversee and participate in the onsite medical pre-screening of invited candidates. SOG anticipates Selection courses to take place bi-annually; however, this could be an annual event based on the needs of the Agency.

6.3 Provide expert physician consultation for the ongoing maintenance of SOG's electronic patient care record and reporting system. SOG Medic patient care records are maintained on a secure USMS server.

6.4 Provide SOG with the necessary subject matter expertise to develop, maintain, and oversee operational medical treatment protocols which shall be provided to all SOG Medics in both hard copy and electronic format. The contractor shall demonstrate at least 15-years' experience developing medical protocols for federal law enforcement EMTs (Basic, Intermediate/Advanced, and Paramedic) in both handbook and electronic format.

- Includes pre-deployment medical briefings and medical threat assessments for domestic and international travel missions/deployments as needed.
- Includes pre-planning, in-transit care plan, and post operational debrief when individuals are transported for official USMS business and accompanied by SOG Medic personnel.

6.5 Contractor shall operate a standing, dedicated hospital-based medical communications center for the purposes of emergency consultations between SOG Medics and physician medical control. The communication center must be staffed twenty-four (24) hours a day, seven (7) days a week by trained emergency communication specialists to handle these calls in a consistent and standard manner. The communications center must possess an intrinsic capability to electronically record all medical consultation calls and maintain the recordings for a minimum of one (1) year.

Contractor shall provide emergency medicine physician 24-hours per day, seven (7) days per week for real-time, on-line and telephone medical consultation with SOG Medic personnel.

SOG Medics must be able to speak to the emergency medicine physician within five (5) minutes of initial contact. Consultations must be through an existing hospital-based communications center that is staffed twenty-four (24) hours a day, seven (7) days a week with communications specialist and dispatch personnel.

SOG aligns its Medic Program practices with the well-established principles and core tenets of EMS medicine, including the essential requirement for physician medical control. Potential reasons for SOG Medics to seek medical control include complex pre-hospital decision support, clarification of protocol-driven interventions, notification of atypical or high complexity situations, and quality assurance purposes. Physicians performing EMS medical control for SOG Medics do not provide direct patient care and do not establish provider-patient relationships. Similarly, SOG does not consider the provision of EMS medical control to constitute the practice of telemedicine.

6.6 Conduct one (1) 40-hour Emergency Medical Technician (EMT) refresher class per year that meets the NREMT requirements. Classes (estimated at approximately 25 students for each class) will either be held at a contractor location or in a SOG specified location. Each EMT refresher class shall include the following:

- 16 hours of continuing education learning, including a CPR recertification course for SOG Medics.
- Instruction will be conducted via indoor and outdoor facilities with space for dynamic tactical medicine scenarios. Training shall accommodate the use of both live and simulated ordinances and be available under both daylight and night-time conditions at either a contractor or SOG facility. Training will be provided by a physician at least 80 percent of the time, and all other instructors shall be tactical medical experts either at a midlevel provider (NP/PA), a registered nurse, or SOG Program Manager (or designee).
- Training and maintenance of Expanded Scope Skills including, but not limited to, airway procedures (oropharyngeal and nasopharyngeal airways, King airway device, I-gel, Endotracheal tube, and surgical cricothyroidotomy); needle procedures (peripheral intravenous access and fluid therapy, intraosseous access and needle thoracostomy); Finger Thoracostomy; medication administration (oral, sublingual and intravenous); hemorrhage control (tourniquet application and topical hemostatic agents); SOLO (Special Operations Low-O)

Damage control resuscitation for non-compressible hemorrhage; and wound care (local anesthetic administration, tissue adhesive application, wound closure-stapling and wound closure-suturing).

- Cadaver lab learning opportunities for anatomy and procedure training, as requested by SOG and if geographically available at/near the refresher class location.
- Provide official EMT refresher class completion documentation to each of the SOG Medic participants (approximately 25 per year) that pass the class, sufficient for NREMT recertification.

6.7 Provide clinical training for SOG Medics in a JHACO-accredited contractor academic medical center that is a Level 1 adult and pediatric trauma center, stroke center, cardiac intervention center, eye trauma center, and EMS Base Station hospital. SOG Medics will have the opportunity to observe and participate in patient care activities including skills training in emergency department, urgent care, and critical care settings. under the supervision of physicians, nurses, and clinical technicians as appropriate.

6.8 Provide deployable Emergency Medicine Physicians in support of remote or high-threat operations and/or training events. Operational support may be required anywhere within the United States and internationally. Training support will occur at various locations throughout the United States to potentially include Hawaii and Alaska.

6.9 Reports and Data: The Contractor shall provide ad hoc reports as requested. From time to time the CO and/or COR may request reports due to data calls from the Department of Justice, USMS senior leadership, auditors and/or inspectors. Data calls for information will be provided by written requests to include description, format, distribution, and due date.

6.10 Contractor shall attend between 6-8 meetings per year. Meeting will be used for planning purposes and to review the SOG Medic Program policies and protocols. At a minimum, all Key Personnel shall attend each meeting.

6.11 Other Personnel Requirements: All physicians performing on this contract must be able to:

- Pass a thorough background investigation, possess a valid state medical license, and be board certified (or eligible) in emergency medicine;
- Must be able to satisfy the requirements for EMS medical direction in accordance with prevailing laws;
- Must be proficient in ultrasounds;
- Physicians must be actively engaged in research involving tactical medicine, and must have been published within the last ten (10) years;
- Physicians must be proficient in the field practice of tactical emergency medicine.

7.0 Key Personnel: The contractor must designate a Medical Director for this contract who can provide physician sanction and licensure, along with training each year of the contract.

As the Medical Director position is designated as “Key Personnel,” all potential Vendors must provide the resume of the individual that will receive this designation. This specific individual’s resume will be used as an evaluation factor in the procurement and any offer can be rejected if there is no firm commitment from the individual listed in the proposal.

Any changes to Key Personnel shall be reported to the CO and COR within twenty-four (24) hours. The notification shall include:

- A brief explanation of the circumstances necessitating the change;
- A complete resume on the proposed replacement (if available at that time);
- Letter of Commitment from the proposed replacement.

The Contractor is required to provide a full resume on the proposed replacement within seventy-two (72) hours to the CO and COR for review. The Government reserves the right to decline any prospective replacement if the requirements outlined in this PWS and the Key Personnel Requirements attachment are not clearly met.

If the proposed replacement is found not qualified by the Government, the Contractor may submit additional information within twenty-four (24) hours to demonstrate that the proposed replacement meets the requirements. If no additional information is provided within the specified time frame, or if the information is insufficient to demonstrate the proposed replacement meets the requirements, then the contractor has forty-eight (48) hours to submit a new proposed replacement. The previous time frame requirements will apply to each proposed replacement until a candidate is found that meets the requirement. The Government reserves the right to terminate the contract for cause should the Contractor not provide an acceptable replacement within ten (10) total days.

*All hour limitations are standard hours – not based on normal “business” hours.

Please see attachment entitled “Key Personnel Requirements” for a list of minimum requirements.

TECHNICAL EXHIBIT 1

Deliverables Schedule

PWS Reference / Deliverable Title	Frequency	Number of Copies	Medium / Format	Submit To
Quality Control Plan (QCP) 2.4	The Contractor's QCP shall be submitted to the COR within 10-days of contract award	1	Electronic Submission	COR
Installation Access and Security Requirements 2.5	The contractor shall provide the names of all contractor personnel who will be servicing this contract to the COR within 10-days of contract acceptance. This is required to get the necessary clearances.	1	Electronic Submission	COR

TECHNICAL EXHIBIT 2

Performance Requirements Summary (PRS)

The PRS includes some of the performance objectives the Government will use to determine Contractor performance to the Acceptable Quality Level (AQL)

Performance Objective	Performance Standard	Acceptable Quality Levels (AQL)	Surveillance Method / By Whom
6.1 Medical Program Oversight	Contractor shall provide medical oversight for the SOG Medic Program	100% Oversight required. Zero Deviation Authorized	Periodic physical & digital inspection / COR
6.1.1 Physician licensure and authorization to carry prescription medications	Contractor shall provide all required licensures and authorizations for SOG Medics to carry prescription medications	100% Required Zero Deviation Authorized	Periodic physical & digital inspection / COR
6.1.2 Training	Contractor shall provide a minimum of 40-hour training per year in TCCC and/or TECC	100% Required Zero Deviation Authorized	Periodic physical & digital inspection / COR
6.2 Provide emergency medical physicians for SOG Course	Contractor shall provide two (2) on-site emergency medical physicians for the first five (5) days of SOG Selection Course	100% Required Zero Deviation Authorized	Periodic physical & digital inspection / COR
6.5 Emergency medicine support	Contractor shall provide emergency medicine support 24-hours/day, 7 days per week.	100% Required Zero Deviation Authorized	Periodic physical & digital inspection / COR

6.6 Conduct 40-hour EMT refresher class	Contractor to conduct one 40-hour EMT refresher class per year.	100% Required Zero Deviation Authorized	Periodic physical & digital inspection / COR
6.8 Provide deployable emergency medicine physicians	Contractor to provide deployable emergency medicine physicians in support of training or high-threat operations	100% Required Zero Deviation Authorized	Periodic physical & digital inspection / COR
6.10 Planning Meetings	Contractor to attend between 6-8 meetings per year in person.	100% Required Zero Deviation Authorized	Periodic physical & digital inspection / COR